

SPMM
Smart
Revise

Epidemiological Data

Paper B

Syllabic content 7.x

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According to ESEMED epidemiological study **1 in 4 adults has lifetime presence of a mental disorder** and **1 in 10 has a mental disorder in last 1 year**. **14.7% had a lifetime history of mood disorder** (major depression only – 13%) while 14% had anxiety (specific phobia only – 8%), and **5.2% had lifetime alcohol use disorder**.

Disorders with higher female preponderance		
Disease	Ratio	Higher in
Social phobia (May be equal in those seeking treatment)	1.2:1	Females
Bipolar disorder type 2	1.3:1	Females
Avoidant personality	1.5:1	Females
Dependent personality	1.5:1	Females
OCD (May be equal in those seeking treatment)	1.5:1	Females
Histrionic personality	1.8:1	Females
Alzheimer's disease (prevalence rates 2:1. Incidence may be equal. More female representation may be due to longevity of lives in women).	2:1	Females
Dysthymia	2:1	Females
Generalised anxiety disorder	2:1	Females
Major depression (this is post-puberty prevalence rates; before puberty, boys marginally > girls)	2:1	Females
Multiple Sclerosis	2:1	Females
PTSD	2:1	Females
Somatization disorder	2:1	Females
Conversion disorder	2-10:1	Females
Panic disorder	2-2.5:1	Females
Agoraphobia	3:1	Females
Borderline personality	3:1	Females
Specific phobias	3:1	Females
Nightmares	2-4:1	Females
Dissociative disorders	4:1	Females
Bulimia nervosa (refer Kaplan & Sadock CTP – quotes new estimates 3:1)	10:1	Females
Anorexia nervosa	10:1	Females

Disorders with higher male preponderance		
Disease	Ratio	Higher in
Lewy body dementia	1.2:1	Males
Paranoid personality disorder	1.3:1	Males
Schizophrenia (Saha et al PLoSMed 2004) incidence rates only. Prevalence is equal.	1.4:1	Males
Parkinson's disease	1.5:1	Males
Conduct Disorder		Males
ADHD	3:1	Males
Alcohol dependence	5:1	Males
Korsakoff's psychosis	2:1	Males
Autism	3:1	Males
Asperger's syndrome Wing 1981 = 10:1	4:1	Males
Antisocial personality disorder	5:1	Males
Tourette's syndrome	4:1	Males

Disorders with equal sex distribution		
Disease	Ratio	Higher in
Anankastic (OCP) personality	1:1	Neither
Frontal Lobe Dementia	1:1	Neither
Huntington's disease (AD inheritance)	1:1	Neither
Schizoid personality disorder	1:1	Neither
Wilson's disease (puzzling 3:1 female preponderance seen for acute liver failure)	1:1	Neither
Bipolar disorder type 1	1:1	Neither

Point estimates

	Point or 1 year Prevalence	Lifetime prevalence	Incidence	Lifetime risk	All cause mortality
Panic disorder	0.9%	4.7%			1.9
Social phobia	2.8%				
Simple phobia	4.8%	7-11%			
Major depressive disorder	5.3%				1.7
Agoraphobia	1.1%	0.8%			
PTSD	1%	8%			
OCD	1.0%	2.5%			1.1
Personality disorders	1.8%				4.0
Schizophrenia(1)	0.33% (1 yr), 0.46% (point)	0.4%	15.2 per 100,000	0.72%	2.6
Bipolar disorder	0.6%	1.5%			2.6
Dementia (>65)	5.4%				2.7
Anorexia	0.37%	1.2-2.2%	4.7-8.3 per 100,000		10
Bulimia	1.5%				
Somatoform disorders	5%*				
Delusional disorders	0.03%		0.7-1.3 per 100,000		

★ All values are in medians; Mortality expressed as SMR. Empty cells reflect inconclusive data

DISCLAIMER: This material is developed from various revision notes assembled while preparing for MRCPsych exams. The content is periodically updated with excerpts from various published sources including peer-reviewed journals, websites, patient information leaflets and books. These sources are cited and acknowledged wherever possible; due to the structure of this material, acknowledgements have not been possible for every passage/fact that is common knowledge in psychiatry. We do not check the accuracy of drug-related information using external sources; no part of these notes should be used as prescribing information

Tables prepared using data from

- ★ Eaton WW, et al. The Burden of Mental Disorders Epidemiol Rev 2008;30:1–14
- ★ European data only; (1) Scz data from McGrath et al.2005.
- ★ ECA data
- ★ Gelder et al., Shorter Oxford textbook of psychiatry
- ★ Also see table 24.2 page 651 of Gelder et al., Shorter Oxford textbook of psychiatry – for childhood disorders.
- ★ Narrow, WE et al (eds). Age and Gender Considerations in Psychiatric Diagnosis: A Research Agenda for DSM-V. American Psychiatric Press, 2007
- ★ Psychol Med. 1998 Jan;28(1):51